Brow Ptosis and Eyelid Repair

Policy Number: MP.002.29
Effective Date: August 1, 2026

➲ Instructions for Use

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Related Commercial/Individual Exchange Policies
- Cosmetic and Reconstructive Procedures
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Community Plan Policy
- Brow Ptosis and Eyelid Repair

Medicare Advantage Policy
- Brow Ptosis and Eyelid Repair

Application

UnitedHealthcare Commercial
This Medical Policy applies to UnitedHealthcare Commercial benefit plans.

UnitedHealthcare Individual Exchange
This Medical Policy applies to Individual Exchange benefit plans.

Coverage Rationale

➲ See Benefit Considerations

Note: The InterQual® criteria below only apply to persons 18 years of age or older.

Brow ptosis repair and repair of the eyelid are considered reconstructive and medically necessary in certain circumstances. For medical necessity clinical coverage criteria, refer to the InterQual® CP: Procedures:
- Blepharoplasty
- Ectropion Repair
- Entropion Repair
- Eyelid Lesion Excision, +/- Reconstruction
- Eyelid Reconstruction
- Ptosis Repair

Click here to view the InterQual® criteria.

Note: If multiple procedures are requested, criteria for each individual procedure must be met.

Internal Browpexy for any condition is considered cosmetic and not medically necessary.

Eyelid surgery for correction of Lagophthalmos is considered reconstructive and medically necessary when the upper eyelid is not providing complete closure to the eye, resulting in dryness and other complications.

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Lid retraction surgery (CPT code 67911) is considered reconstructive and medically necessary for conditions other than thyroid eye disease when all the following criteria are present:

- Other causes have been eliminated as the reason for the lid retraction such as use of dilating eye drops or glaucoma medications; and
- There is a functional impairment (e.g., dry eyes, pain/discomfort, tearing, blurred vision); and
- Tried and failed conservative treatments

For thyroid eye disease, lid retraction surgery (CPT code 67911) is considered reconstructive and medically necessary, when either of the following criteria are met:

- Moderate to severe disease where all the following apply:
  - The disease is stable or inactive, and euthyroidism has been achieved and maintained; and
  - Other causes of lid retraction have been eliminated (e.g., use of dilating eye drops or glaucoma medications); and
  - There is a functional impairment (e.g., dry eyes, pain/discomfort, tearing, blurred vision)

  or
- Severe, sight-threatening thyroid eye disease to address subluxation

Canthoplasty/Canthopexy (CPT codes 21280, 21282, and 67950) are considered reconstructive and medically necessary when all the following criteria are present:

- There is a functional impairment; and
- Repair of ectropion or entropion will not correct condition; and
- At least one of the following is present:
  - Epiphora (excess tearing) not resolved by conservative measures; or
  - Corneal dryness unresponsive to lubricants; or
  - Corneal ulcer

Repair of Floppy Eyelid Syndrome (CPT codes 67961 and 67966) is considered reconstructive and medically necessary when all the following are present and have been documented and confirmed by history and examination:

- Subjective symptoms must include eyelids spontaneously "flipping over" when the member sleeps due to rubbing on the pillow and one of the following:
  - Eye pain or discomfort; or
  - Excess tearing; or
  - Eye irritation, ocular redness, and discharge
- Physical examination that documents all the following:
  - Both of the following:
    - Eyelash ptosis; and
    - Significant upper eyelid laxity

    and
  - One of the following:
    - Presence of giant papillary conjunctivitis; or
    - Corneal findings such as one of the following:
      - Superficial punctate erosions; or
      - Corneal abrasion (documentation of a history of corneal abrasion or recurrent erosion syndrome is considered sufficient); or
      - Microbial keratitis
- Clear, high-quality, clinical photographs that clearly document Floppy Eyelid Syndrome and demonstrate both of the following:
  - Lids must be everted in the photographs; and
  - Conjunctival surface (underbelly) of the lids must be clearly visible
- Documentation that conservative treatment has been tried and failed; examples may include:
  - Ocular lubricants both drops (daytime) and ointments (bedtime)
  - Short trial of antihistamines
  - Topical steroid drops
  - Eye shield and/or taping the lids at bedtime
- Infections of the eye have been ruled out; examples may include:
  - Allergic conjunctivitis
  - Atopic keratoconjunctivitis
  - Blepharitis
  - Contact lens complication (e.g., giant papillary conjunctivitis)

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- Superior limbic keratoconjunctivitis

Medical Records Documentation Used for Reviews

Benefit coverage for health services is determined by the member specific benefit plan document and applicable laws that may require coverage for a specific service. Medical records documentation may be required to assess whether the member meets the clinical criteria for coverage but does not guarantee coverage of the service requested; refer to the guidelines titled Medical Records Documentation Used for Reviews.

Definitions

The following definitions may not apply to all plans. Refer to the member specific benefit plan document for applicable definitions.

Canthopexy: A surgical technique for lid malposition that involves securing the lateral retinaculum to the periosteum of the superolateral orbital rim with a suture (Rizvi et al., 2010).

Canthoplasty: A procedure that is indicated for a variety of eyelid conditions. It is applicable to any disruption to the normal architecture of the canthus, which can lead to negative functional sequelae (American Academy of Ophthalmology, 2023).

Floppy Eyelid Syndrome: A frequent eyelid disorder characterized by eyelid laxity that determines a spontaneous eyelid eversion during sleep associated with chronic papillary conjunctivitis and systemic diseases (De Gregorio et al., 2021).

Internal Browpexy: A minimally invasive technique to provide stabilization and subtle elevation of the lateral brow (Karimi et al., 2020).

Lagophthalmos: The inability to close the eyelids completely. A portion of the eye remains open during a blink and during sleep and is subject to damage from exposure (American Academy of Ophthalmology Exposure Keratopathy, 2025).

Applicable Codes

The following list(s) of procedure and/or diagnosis codes is provided for reference purposes only and may not be all inclusive. Listing of a code in this policy does not imply that the service described by the code is a covered or non-covered health service. Benefit coverage for health services is determined by the member specific benefit plan document and applicable laws that may require coverage for a specific service. The inclusion of a code does not imply any right to reimbursement or guarantee claim payment. Other policies and guidelines may apply.

Note: The following codes may be cosmetic; review is required to determine if considered cosmetic or reconstructive.

CPT Code	Description
Blepharoplasty (Lower and Upper Eyelid)	
15820	Blepharoplasty, lower eyelid
15821	Blepharoplasty, lower eyelid; with extensive herniated fat pad
15822	Blepharoplasty, upper eyelid;
15823	Blepharoplasty, upper eyelid; with excessive skin weighting down lid
Brow Ptosis Repair	
67900	Repair of brow ptosis (supraciliary, mid-forehead or coronal approach)
Upper Eyelid Blepharoptosis Repair	
67901	Repair of blepharoptosis; frontalis muscle technique with suture or other material (e.g., banked fascia)
67902	Repair of blepharoptosis; frontalis muscle technique with autologous fascial sling (includes obtaining fascia)
67903	Repair of blepharoptosis; (tarso) levator resection or advancement, internal approach
67904	Repair of blepharoptosis; (tarso) levator resection or advancement, external approach

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CPT Code	Description
Upper Eyelid Blepharoptosis Repair	
67906	Repair of blepharoptosis; superior rectus technique with fascial sling (includes obtaining fascia)
67908	Repair of blepharoptosis; conjunctivo-tarso-Muller's muscle-levator resection (e.g., Fasanella-Servat type)
67909	Reduction of overcorrection of ptosis
Lid Retraction	
67911	Correction of lid retraction
Lagophthalmos	
67912	Correction of lagophthalmos, with implantation of upper eyelid lid load (e.g., gold weight)
Ectropion and Entropion	
67914	Repair of ectropion; suture
67915	Repair of ectropion; thermocauterization
67916	Repair of ectropion; excision tarsal wedge
67917	Repair of ectropion; extensive (e.g., tarsal strip operations)
67921	Repair of entropion; suture
67922	Repair of entropion; thermocauterization
67923	Repair of entropion; excision tarsal wedge
67924	Repair of entropion; extensive (e.g., tarsal strip or capsulopalpebral fascia repairs operation)
Canthoplasty/Canthopexy	
21280	Medial canthopexy (separate procedure)
21282	Lateral canthopexy
67950	Canthoplasty (reconstruction of canthus)
67961	Excision and repair of eyelid, involving lid margin, tarsus, conjunctiva, canthus, or full thickness, may include preparation for skin graft or pedicle flap with adjacent tissue transfer or rearrangement; up to one-fourth of lid margin
67966	Excision and repair of eyelid, involving lid margin, tarsus, conjunctiva, canthus, or full thickness, may include preparation for skin graft or pedicle flap with adjacent tissue transfer or rearrangement; over one-fourth of lid margin
Floppy Eyelid Syndrome	
67961	Excision and repair of eyelid, involving lid margin, tarsus, conjunctiva, canthus, or full thickness, may include preparation for skin graft or pedicle flap with adjacent tissue transfer or rearrangement; up to one-fourth of lid margin
67966	Excision and repair of eyelid, involving lid margin, tarsus, conjunctiva, canthus, or full thickness, may include preparation for skin graft or pedicle flap with adjacent tissue transfer or rearrangement; over one-fourth of lid margin

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Benefit Considerations

Some states require benefit coverage for services that UnitedHealthcare considers cosmetic procedures, such as repair of external congenital anomalies in the absence of a functional impairment. Refer to the member specific benefit plan document.

UnitedHealthcare excludes cosmetic procedures from coverage, including but not limited to the following:

- Procedures that correct an anatomical congenital anomaly without improving or restoring physiological function are considered cosmetic procedures. The fact that a covered person may experience psychological consequences or socially avoidant behavior as a result of an injury, sickness, or congenital anomaly does not classify surgery (or other procedures done to relieve such consequences or behavior) as a reconstructive procedure.

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Clinical Evidence

Internal Browpexy

An internal browpexy is best for mild brow ptosis and is considered cosmetic. It is difficult to generate a substantial amount of elevation using this technique (Shaw and Phelps, 2020).

Huang et al. (2023) conducted an objective comparison of eyebrow position following internal and external browpexy by retrospectively reviewing the cases of 68 patients, who were mostly female, who underwent either internal browpexy (39), external browpexy (nine), or upper eyelid skin excision alone (20). Photographs were taken before surgery, immediately after surgery, and then at 1 week, 1 to 3 months, 4 to 6 months, and 7 to 12 months. Brow height changes were measured using the ImageJ biological measuring program and compared among the three groups. The results showed that both internal and external browpexy provided improvement within 3 months of surgery; however, outcomes in those treated with an external browpexy had better brow-lift outcomes across the entire brow than those treated with internal browpexy. The authors concluded that external browpexy is a better choice for those with severe whole-brow ptosis, and internal browpexy is recommended for individuals with mild ptosis. This retrospective observational study is limited by a small number of patients and a lack of randomization. Furthermore, most patients were lost to follow-up, so longer-term data are not available. Further high-quality research is needed to validate these findings.

Floppy Eyelid Syndrome

There is no diagnostic test for floppy eyelid syndrome (FES). It is a clinical diagnosis based on history and symptoms and is a sequela of systemic conditions such as obstructive sleep apnea (OSA) and morbid obesity. The eye disorder keratoconus may also contribute to FES. Individuals with this condition should be managed by an internal medicine team and a sleep disorder specialist. Addressing obesity and OSA as well as avoiding sleeping in a prone position may improve symptoms. If there is minimal response to these interventions, surgical procedures such as horizontal eyelid shortening can help relieve ocular symptoms and provide good functional and cosmetic results. Surgery should be considered in significantly symptomatic individuals after controlling ocular surface disease and optimizing medical status [American Academy of Ophthalmology (AAO)].

Additional systematic review and meta-analyses have explored the association of FES and OSA (Aiello et al., 2023; Bulloch et al., 2023) and concluded that there is an association between the two, with the severity of OSA correlating with a higher incidence of FES.

Cheong et al. (2023) conducted a systematic review and meta-analysis to investigate the relationship between OSA and FES. The systematic review included 12 studies, nine of which were included in the meta-analysis, with a total of 1,109 individuals. The analysis of the data determined a significant association between OSA and FES [odds ratio (OR), 1.89; 95% CI, 1.27-2.83; I^2 = 44%]. On further investigation, the study determined that the more severe the OSA, the higher the risk of developing FES. Individuals with severe OSA had the highest risk of developing FES (OR, 3.06; 95% CI, 1.62-5.78; I^2 = 0%), followed by moderate OSA (OR, 2.53; 95% CI, 1.29-4.97; I^2 = 0%); those with mild OSA had the lowest risk (OR, 1.76; 95% CI, 0.85-3.62; I^2 = 0%). The authors concluded that there was a positive association between OSA and FES, with increasing severity of OSA correlating with a significantly higher risk of FES. A limitation in the study is that important covariates such as age, gender, and body mass index were not adjusted. The authors recommended more longitudinal studies, with a sufficient duration of follow-up, to better characterize the relationship between OSA and FES.

Acar et al. (2021) conducted a randomized controlled trial in 51 participants with obstructive sleep apnea-hypopnea syndrome to assess the long-term effects of positive airway pressure (PAP) therapy on the eyelid and the ocular surface. Over a period of 18 months, participants were treated with PAP, and the scores were compared for the pre- and post-PAP values for eye examination, which included the presence of FES, Ocular Surface Disease Index questionnaire results, Schirmer I test, tear film breakup time, and corneal staining. The presence of FES before and after PAP was 56.9% and 74.5% ($p < 0.01$). FES stage was determined to be 1.41 ±0.98 before PAP and 0.78 ±0.78 after PAP ($p < 0.01$). Pre-PAP and post-PAP Ocular Surface Disease Index results were 47.79 ±21.04 and 42.17 ±19.97 ($p < 0.01$). Schirmer values before and after PAP were 7.23 ±1.95 and 8.49 ±1.79 mm ($p < 0.01$). Tear film breakup time values before and after PAP were 7.11 ±1.82 and 8.68 ±1.76 seconds ($p < 0.01$). Scores of the corneal staining stages before and after PAP were 1.05 ±0.75 and 0.68 ±0.54 ($p < 0.01$). The authors concluded that obstructive sleep apnea-hypopnea syndrome was associated with low Schirmer and tear film breakup time values, high scores on the Ocular Surface Disease Index questionnaire, and high corneal staining. Normal sleep patterns returned after appropriate use of PAP, in addition to relief of systemic findings and ocular surface problems. The authors believe that long-term use of PAP (at least 1 year) improves FES and overcomes the problem of ocular irritation that occurs in the early stage of PAP therapy. Limitations of the study include the lack of blinding when performing the ocular screenings and a small sample size.

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Lagophthalmos

Proper eyelid closure and a normal blink reflex are essential to maintaining a stable tear film and a healthy corneal surface. Individuals who experience lagophthalmos are unable to fully close their eyelids and may describe symptoms of dry and irritated eyes. Common morbidities of lagophthalmos are corneal exposure and subsequent keratopathy, which may progress to corneal ulceration and infectious keratitis. It is important to recognize lagophthalmos early and begin treatment as soon as possible. The choice of therapy requires an understanding of both the etiology and expected duration of the lagophthalmos. The etiology of lagophthalmos is due to damage to the facial nerve, which can be from trauma, stroke, Bell palsy, tumors, Möbius syndrome, and infections or immune-mediated causes. Medical treatment and supportive care include artificial tears and ointments, moisture goggles, and patches. Surgical treatments include tarsorrhaphy and the implantation of gold weights; upper and lower eyelid retraction; upper eyelid levator muscle recession; and lower eyelid elevation. Treatment is done in a stepwise approach based on the severity and duration of the condition (AAO, 2008).

Ottonelli et al. (2025) conducted a systematic review of 26 studies, totaling 1,275 individuals who underwent surgical management for upper eyelid retraction and lagophthalmos due to facial nerve palsy. Studies were limited to static upper lid methods that augment mass or volume and included upper eyelid weight implantation (most common), autologous fat grafting, and müllerectomy in individuals with symptomatic lagophthalmos, ocular surface symptoms, or cosmetic disfigurement. Excluded were studies that were unrelated to nerve palsy, those that focused on nonsurgical treatments or lower eyelid procedures, case reports, case series with fewer than 10 individuals, animal and cadaver studies, and those with a lack of relevant outcome data reported. The underlying etiologies were idiopathic facial nerve palsy (Bell palsy); postsurgical and posttrauma; and, less frequently, congenital. The study size ranged from 10 to 130 individuals, who ranged in age from 6 to 92 years, with an average age in the middle fifties. Procedures were unilateral and bilateral. Follow-up ranged from 2 months to 10 years, with a median of 12 to 48 months. Of the 26 included studies, 23 addressed the use of gold or platinum eyelid weights, and all results showed consistent improvement in eye closure; the patient-reported outcomes were favorable. Adverse events were uncommon and included implant extrusion and migration and related skin ulceration and infection. Two studies investigated autologous fat grafting to treat paralytic lagophthalmos in 85 individuals. In one study in 75 individuals, the results showed that 79% experienced a persistent benefit, and 11.6% reported only transient or no improvement; repeat procedures were needed in 8.7%. A small cohort of 10 individuals had a high degree of satisfaction, with 60% discontinuing the use of artificial tears or ointments. Two individuals required repeat procedures. In the one study that addressed müllerectomy, 18 received müllerectomy only (no additional lower eyelid surgery), and the results showed significant improvement, with a mean reduction of 1.8 mm ($p = 0.002$), complete or near-complete resolution of keratopathy, and improvement in corneal exposure and superficial punctate keratopathy. The authors concluded that upper eyelid weight implantation shows the highest level of evidence and consistency, with improvements ranging from 83% to 92% across large cohorts. This systematic review is limited by the study design (single-center, retrospective case series); no studies were randomized. Furthermore, there was extensive heterogeneity in the selection of individuals, surgical details, follow-up intervals, and outcome definitions. Prospective randomized studies that include larger populations of individuals are needed that address these limitations.

Lid Retraction Surgery

Upper eyelid retraction is defined by an abnormally high resting position of the upper lid. This produces visible sclera between the eyelid margin and corneal limbus, which produces the appearance of a stare with an accompanying illusion of exophthalmos. Eyelid retraction can lead to lagophthalmos and exposure keratitis, which can cause mild ocular surface irritation to vision-threatening corneal decompensation. The most common cause is thyroid eye disease (TED). It may also be congenital, neurogenic, or myogenic in nature. Mild upper eyelid retraction in TED can resolve spontaneously over time; if it does not or the condition is causing an immediate threat to the cornea or vision, tarsorrhaphies or recession of the upper eyelid is a surgical treatment. Other treatments, when surgery is not an option, include transconjunctival botulinum toxin A injections, triamcinolone acetonide deep fornix and subconjunctival injections, and hyaluronic acid filler subconjunctival injections (AAO, 2025).

Lower eyelid retraction is a malposition of the lower eyelid in which the lid margin is displaced inferiorly, resulting in increased exposure of the surface of the eye to the environment. It is seen unilaterally and bilaterally, depending on the etiology. The condition most frequently presents due to TED; however, it can also be caused by other myogenic, neurogenic, mechanical, or congenital conditions and can be unilateral or bilateral, depending on the etiology. It can present with dry eye symptoms and may lead to exposure keratitis, corneal ulcer, scarring, or perforation if not managed. Mild cases may be managed with ocular surface lubrication, and more severe cases may be managed with grafting. (AAO, 2025).

Medial and Lateral Canthoplasty/Canthopexy

The AAO states that the following are indications for functional canthoplasty:

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- Congenital and involutional entropion
- Congenital, involutional, and cicatricial ectropion
- Lid laxity (seen with anophthalmos or enophthalmos and facial nerve palsy)
- Canthal dystopia
- Exposure keratopathy
- Epiphora
- Vertical eyelid retraction [due to trauma, after blepharoplasty, with TED (contraindicated if significant proptosis)]
- Repair after iatrogenic damage or trauma
- In conjunction with blepharoplasty:
  - To prevent ectropion or eyelid retraction
  - Festoons
- Delayed repair, resulting in rounding of the canthus
- Telecanthus
- Congenital malposition or occlusion of the visual field
- Absent naso-orbital valley
- With lateral orbitotomy:
  - In orbital decompression
  - Removal of orbital tumors

U.S. Food and Drug Administration (FDA)

This section is to be used for informational purposes only. FDA approval alone is not a basis for coverage.

Brow ptosis repair and eyelid repair are procedures and therefore are not regulated by the FDA. However, devices and instruments used during the surgery may require FDA approval. Refer to the following website for additional information: http://www.accessdata.fda.gov/scripts/cdrh/cfdocs/cfPMN/pmn.cfm. (Accessed February 17, 2026)

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Policy History/Revision Information

Date	Summary of Changes
08/01/2026	**Coverage Rationale** • Replaced language indicating “lid retraction surgery (CPT code 67911) is considered reconstructive and medically necessary when all of the [listed] criteria are present” with “lid retraction surgery (CPT code 67911) is considered reconstructive and medically necessary *for conditions other than thyroid eye disease* when all the [listed] criteria are present” ***Thyroid Eye Disease*** • Revised coverage guidelines to indicate lid retraction surgery (CPT code 67911) for thyroid eye disease is considered reconstructive and medically necessary when either of the following criteria are met: ○ Moderate to severe disease where all the following apply: ▪ The disease is stable or inactive, and euthyroidism has been achieved and maintained ▪ Other causes of lid retraction have been eliminated (e.g., use of dilating eye drops or glaucoma medications) ▪ There is a functional impairment (e.g., dry eyes, pain/discomfort, tearing, blurred vision) ○ Severe, sight-threatening thyroid eye disease to address subluxation **Definitions** • Removed definition of “Marginal Reflex Distance -1 (MRD-1)” **Supporting Information** • Updated *Clinical Evidence* and *References* sections to reflect the most current information • Archived previous policy version MP.002.28

Instructions for Use

This Medical Policy provides assistance in interpreting UnitedHealthcare standard benefit plans. When deciding coverage, the member specific benefit plan document must be referenced as the terms of the member specific benefit plan may

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differ from the standard plan. In the event of a conflict, the member specific benefit plan document governs. Before using this policy, check the member specific benefit plan document and any applicable federal or state mandates. UnitedHealthcare reserves the right to modify its policies and guidelines, as necessary. This Medical Policy is provided for informational purposes. It does not constitute medical advice.

This Medical Policy may also be applied to Medicare Advantage plans in certain instances. In the absence of a Medicare National Coverage Determination (NCD), Local Coverage Determination (LCD), or other Medicare coverage guidance, CMS allows a Medicare Advantage Organization (MAO) to create its own coverage determinations, using objective evidence-based rationale relying on authoritative evidence (Medicare IOM Pub. No. 100-16, Ch. 4, §90.5).

UnitedHealthcare may also use tools developed by third parties, such as the InterQual® criteria, to assist us in administering health benefits. UnitedHealthcare Medical Policies are intended to be used in connection with the independent professional medical judgment of a qualified health care provider and do not constitute the practice of medicine or medical advice.

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